

Apollo Hospitals Chennai

21, Greams Lane, Off Greams Road, Chennai - 600006, Tamil Nadu, India

Tel: +91-44-2829-3333 | Reg No: TN-MED-2004-00312

PATIENT DISCHARGE SUMMARY & CLINICAL RECORD

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SECTION 1 — PATIENT IDENTIFICATION

Patient Full Name	Patient ID (MRN)
Rajesh Kumar Venkataraman	APL-CH-2024-08821
Date of Birth	Age
14/03/1978	46 years
Sex	Blood Group
Male	B+
Residential Address	
47, 3rd Cross Street, Thiruvanmiyur, Chennai - 600041, Tamil Nadu, India	
Contact Number	Emergency Contact
+91-98400-12345	Meena Venkataraman (+91-98400-67890)
Insurance ID	Nationality
STAR-HLTH-TN-4492310	Indian

SECTION 2 — ADMISSION DETAILS

Admission Date	Discharge Date
05/09/2024	11/09/2024
Ward / Unit	Attending Physician
Cardiology ICU — Bed 4B	Dr. Suresh Nair (MD, DM Cardiology)
Referring Physician	Department
Dr. Priya Anand (General Physician)	Cardiology
Reason for Admission	
Acute chest pain with radiation to left arm, onset 6 hours prior to admission. History of hypertension.	

SECTION 3 — PRESENTING SYMPTOMS

- Acute chest pain, substernal, radiating to left arm (onset ~6 hours)
- Diaphoresis (profuse sweating)
- Nausea without vomiting
- Shortness of breath on exertion
- Dizziness and mild lightheadedness

Symptom Onset	Duration
05/09/2024 at 08:30 AM	6 hours prior to admission

SECTION 4 — DIAGNOSIS

Primary Diagnosis

ST-Elevation Myocardial Infarction (STEMI) — Anterior wall

Secondary Diagnosis

Hypertension (Grade 2), Type 2 Diabetes Mellitus

ICD-10 Code

SECTION 5 — LABORATORY RESULTS

Marker	Value	Unit	Reference Range	Flag
Troponin I	18.4	ng/mL	< 0.04	HIGH
CK-MB	112	U/L	0–25	HIGH
WBC	13,200	cells/uL	4,500–11,000	HIGH
Hemoglobin	12.8	g/dL	13.5–17.5	LOW
Platelet Count	210,000	cells/uL	150,000–400,000	NORMAL
Creatinine	1.3	mg/dL	0.7–1.2	HIGH
HbA1c	8.2	%	< 5.7	HIGH
LDL Cholesterol	178	mg/dL	< 100	HIGH
Blood Glucose (F)	198	mg/dL	70–100	HIGH

SECTION 6 — MEDICATIONS ADMINISTERED

Medication	Dosage	Frequency	Response
Aspirin	325 mg	Once daily	Good response
Clopidogrel	75 mg	Once daily	Good response
Metoprolol	25 mg	Twice daily	Partial response
Atorvastatin	80 mg	Once at night	Good response
Ramipril	5 mg	Once daily	Good response
Insulin Glargine	10 units	Once at bedtime	Moderate response

SECTION 7 — PROCEDURES & IMAGING

- 12-lead ECG — ST elevation in leads V1-V4 confirmed
- Emergency Coronary Angiography — LAD occlusion identified
- Primary PCI (Percutaneous Coronary Intervention) — Stent placed in LAD
- Echocardiography — EF 42%, anterior wall hypokinesia
- Continuous cardiac monitoring (6 days)

Imaging Metadata

Chest X-ray (05/09/2024): Mild cardiomegaly, no pulmonary edema. Echo (06/09/2024): EF 42%, anterior wall motion abnormality. Coronary Angiogram (05/09/2024): 100% LAD occlusion at proximal segment. Post-PCI Echo (09/09/2024): EF improved to 48%.

SECTION 8 — GENOMIC INFORMATION (Optional)

Genomic Findings

APOE e3/e4 variant detected — elevated cardiovascular risk. No BRCA variants. CYP2C19 *2/*1 — clopidogrel intermediate metabolizer.

SECTION 9 — CLINICAL OUTCOME

Discharge Outcome	Follow-up Required
Recovered — Stable	Yes — Cardiology OPD in 2 weeks
Discharge Instructions	

Strict medication adherence. Low-fat, low-sodium diet. Avoid strenuous activity for 4 weeks. Daily blood glucose monitoring. Return immediately if chest pain recurs. Cardiac rehab referral given.

SECTION 10 — CLINICAL NOTES

Physician Clinical Notes

46-year-old male with known hypertension and T2DM presented with acute STEMI. Emergency cath lab activated. Primary PCI performed within 90 minutes of arrival. Drug-eluting stent placed in proximal LAD. Post-procedure troponin trending down. EF improved from 42% to 48% by Day 4. Patient counselled on lifestyle modification, smoking cessation (patient is non-smoker), dietary changes. Discharged stable on Day 6 with dual antiplatelet therapy and statin. Follow-up arranged with cardiology in 2 weeks.

Signed by: Dr. Suresh Nair (MD, DM Cardiology) | Date: 11/09/2024 | Document ID: APL-CH-2024-08821-DS-2024
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